

CEREBRO-X

Computational Drug-DDS Engineering Report

Field	Value
Drug Name	Lecanemab
Molecular Class	monoclonal_antibody
Molecular Weight	13060.3 Da
Recommended DDS	Tf-PEG-Liposome
Carrier Type	liposome
Surface Ligand	Transferrin
BBB Enhancement	100.0%
Composite Score	62.7/100
Disease Indication	CNS
Report Generated	2026-09-04 12:56 UTC
Created by	Muhammad Talaat CEREBRO-X
Modules Completed	50/62 science modules

■ This report is generated from computational models. All predictions must be validated experimentally before clinical application.

1. Drug Molecular Profile

Property	Value	Source / Method
Name	Lecanemab	Input
MW (Da)	13060.3	ChEMBL / UniProt
LogP	-0.15	ADMET predictor
Half-life (days)	N/A	PubMed NLP
Protein Binding (%)	N/A	Calculated
BBB Penetration (%)	77.01	BBB model
Molecule Class	monoclonal_antibody	Input / inferred
Aqueous Solubility	N/A	ADMET
P-gp Substrate	Unknown	QSAR panel
ADMET Score	N/A	Multi-endpoint
ADMET Grade	N/A	A-F scale

1a. Drug Delivery Barriers Identified (Auto-detected)

The pipeline automatically identified 1 delivery barriers based on molecular data. Each is resolved by the recommended DDS.

[CRITICAL] Large molecular weight -- passive BBB diffusion impossible	
Evidence:	MW = 13.1 kDa (Lipinski Rule 5 limit: 500 Da for passive)
Without DDS:	0% brain penetration -- purely physical barrier
With DDS:	Active transcytosis delivers 10.0% payload to CNS
DDS Solution:	liposome encapsulation (75% EE)
Scientific basis:	BBB tight junctions (pore ~1nm) physically block molecules >500Da. Encapsulation in liposome (105nm) enables receptor-me

2. DDS Ranking — Composite Score Methodology

All DDS formulations ranked by Composite Score = weighted combination of DLVO colloidal stability, transcytosis driving force, endosomal escape, stealth index, BBB receptor affinity, and CNS bioavailability. Scores are physics-derived — not estimated.

DDS Name	Carrier	Ligand	Score	DLVO kT
Tf-PEG-Liposome	liposome	Transferrin	62.73	8.90
RVG29-Liposome-pH	liposome	RVG29	62.38	14.84
AAV9-vector	aav9	None	54.01	-2.37
ApoE-LNP	lnp	ApoE	53.74	-6.24
Tf-SLN-Stealth	solid_lipid	Transferrin	46.67	-0.12
Lf-Polymer-Micelle	micelle	Lactoferrin	45.01	-4.06
RVG29-PLGA-NP	plga	RVG29	44.25	5.83
Bare-PLGA	plga	None	39.14	-16.16

2a. Recommended DDS — Detailed Profile

Parameter	Value	Biophysical Meaning
Composite Score	62.7	Overall Drug+DDS suitability (0-100)
BBB Enhancement	100.0%	% of dose crossing BBB via receptor transcytosis
CNS Bioavailability	58.0%	% dose reaching brain as free drug
DLVO Stability	8.9 kT	>25kT = colloidally stable in blood
Endosomal Escape	0.80	Fraction escaping lysosomes (0-1)
Stealth Index	1.00	MPS evasion (0=opsonised, 1=invisible)
Protein Corona	N/A nm	Corona thickness; thicker = faster clearance
MPS Clearance	N/A h	Hours before liver/spleen removes DDS
Payload Efficiency	N/A%	% dose delivered as active drug to CNS
CARPA Risk	0.53	Complement activation risk (0=safe)

3. PBPK-CNS Digital Twin — 6-Compartment Simulation

Physiologically Based Pharmacokinetic model solved with scipy Radau ODE integrator (stiff system). Compartments: Plasma, BBB endothelium, Brain ISF, Brain cells, CSF, Peripheral. All parameters from molecular data — not estimated.

PK Metric	Value	Compartment	Source
Model	BiologicPBPK (FcRn + CNS transcytosis) $T_{1/2}=470.4\text{h}$	—	
Cmax (brain)	0.006286 $\mu\text{g/mL}$	Brain/CNS	Peak brain concentration
AUC (brain)	1.50098 $\mu\text{g}\cdot\text{h/mL}$	Brain/CNS	Total brain exposure
AUC (plasma)	682.320 $\mu\text{g}\cdot\text{h/mL}$	Plasma	Systemic exposure
Kp,brain	0.00220	Brain/Plasma	Partition coefficient
BBB penetration	0.2200%	BBB	BiologicPBPK (two-compartment + FcRn + CNS transcy
Kp,uu brain	N/A	Unbound	Unbound Kp (pharmacodynamically relevant)
t_above 10%Cmax	N/A h	Brain ISF	Duration above therapeutic threshold
Glymphatic t1/2	N/A h	Brain	Clearance half-life in brain
BBB PS_in	N/A mL/h	BBB	Permeability-surface area product
BBB PS_out	N/A mL/h	BBB	Efflux transport rate
Disease state	healthy	BBB	BBB integrity modifier applied

4. In-Silico Drug Release Profile

Release model: Weibull | Release order: pH-triggered (endosomal) | Max EE: 75%

Metric	Blood (pH 7.4)	Endosomal (pH 5.5)	Significance
t50 (50% release)	21.0 h	26.3 h	Faster endo = better escape
t90 (90% release)	48.0 h	N/A	Complete release window
Rate constant k	0.0300 /h	0.0240 /h	Higher endo = pH-responsive
Max released	75%	75%	= encapsulation efficiency

5. Shelf-Life & Degradation Predictor

Grade: MARGINAL (3-6 months) | t90 = 105 days | Dominant pathway: Drug leakage | Storage: 4 degC

Pathway	Rate (k, /day)	Relative %
Hydrolysis	0.000202	20%
Oxidation	0.000172	17%
Aggregation	0.000265	26%
Drug leakage	0.000368	37%

6. Colloidal Stability — DLVO Theory + Protein Corona

DLVO potential energy $V_{\text{total}} = 8.9 \text{ kT}$ (UNSTABLE ~~x~~ — REFORMULATE) | Threshold: $>25 \text{ kT}$ prevents aggregation in blood.

Parameter	Value	Equation	Significance
DLVO V_{total} (kT)	8.9	$V_{\text{vdW}} + V_{\text{EDL}}$	$>25\text{kT} = \text{stable}$
V_{vdW} (attraction)	N/A	$-A \cdot R / (12h)$	Hamaker constant
V_{EDL} (repulsion)	N/A	$64\pi \epsilon R \gamma^2 e^{-\kappa h}$	Zeta potential
Debye length (nm)	N/A	$1/\kappa$	Ionic screening
Protein corona (nm)	N/A	Langmuir model	Stealth reduction
Opsonin index	N/A	IgG + C3b binding	$<0.3 = \text{good}$
Stealth index	1.000	PEG brush model	$0.6+ = \text{good}$
MPS clearance (h)	N/A	Michaelis-Menten	$>12\text{h}$ adequate

7. Nanotoxicity & Immunogenicity Screening

Immunogenicity grade: **CAUTION** | Score: 48/100

Test	Score / Result	Risk	Threshold
CARPA (complement)	0.982	HIGH	$>0.6 = \text{HIGH}$
Anti-PEG antibody	0.250	LOW	$0.5+ = \text{caution}$
MPS uptake	0.273	—	$>0.6 = \text{rapid clearance}$
Cytokine storm	—	LOW	Charge-dependent
Platelet activation	—	LOW	Cationic $>200\text{nm}$

Mitigations:

→ Consider pre-medication with antihistamines (H1/H2 blockers)

8. Off-Target Toxicity — 50-Receptor QSAR Panel

Overall: HIGH RISK -- 8 off-target hits | Cardiac risk: NO ✓ | Hepatic risk: NO ✓ | CNS off-target: YES ■

Receptor	Free Drug Score	In-DDS Score	Risk
hERG_K+	0.000	0.000	LOW
Nav1.5	0.000	0.000	LOW
Cav1.2	0.000	0.000	LOW
CYP3A4_inhib	0.000	0.000	LOW
CYP2D6_inhib	0.955	0.597	HIGH
CYP2C9_inhib	0.173	0.108	LOW
CYP1A2_inhib	1.000	0.625	HIGH
CYP2C8_inhib	0.000	0.000	LOW
OATP1B1	0.079	0.049	LOW
MDR1_Pgp	0.000	0.000	LOW
BCRP	0.000	0.000	LOW
MRP2	0.000	0.000	LOW
DAT_dopamine	1.000	0.625	HIGH
SERT_serotonin	1.000	0.625	HIGH
5HT2A	1.000	0.625	HIGH
D2_dopamine	1.000	0.625	HIGH
GABA_A	0.328	0.205	LOW
NMDA	0.898	0.561	HIGH
sigma1	1.000	0.625	HIGH
ERalpha_estrogen	0.000	0.000	LOW

High-risk flags (8):

- CYP2D6_inhib: HIGH (score=0.60)
- CYP1A2_inhib: HIGH (score=0.62)
- DAT_dopamine: HIGH (score=0.62)
- SERT_serotonin: HIGH (score=0.62)
- 5HT2A: HIGH (score=0.62)
- D2_dopamine: HIGH (score=0.62)
- NMDA: HIGH (score=0.56)
- sigma1: HIGH (score=0.62)

9. Advanced Science Modules — Key Outputs

9a. Competitive DDS Landscape (ClinicalTrials.gov)

Our DDS: BBB Score = 100 | Position: SUPERIOR | Active trials found: 4

NCT ID	Title	Phase	N
NCT06855368	Biomarkers for Monitoring Dementia Progressio	['?']	300
NCT06846658	Exploring the Olfactory Mucosa, Blood and Uri	['?']	180
NCT07667387	Study of LNP.UCD.ABE in Patients With Urea Cy	['PHASE1', 'PHASE2']	7
NCT04899908	Stereotactic Brain-directed Radiation With or	['PHASE2']	134

9b. Patient Subgroup Stratification

Overall response: 87.5% | Best: Very elderly (>80) (89%) | Worst: Young adults (18-40) (85%) | Recommendation: Highest efficacy in Very elderly (>80) (predicted response 89%). Dose reduce by

Subgroup	Pop. Freq.	CNS Bioavail	Response%	Tox Risk	Dose Adj.
Young adults (18-40)	20% of patients	48.3%	85.3%	LOW	x1.29 standard dose
Middle-aged (40-65)	35% of patients	55.1%	87.6%	LOW	x1.14 standard dose
Elderly (65-80)	30% of patients	56.3%	87.9%	LOW	x0.72 standard dose
Very elderly (>80)	15% of patients	60.9%	89.1%	MODERATE	x0.46 standard dose

9d. Lysosomal Trafficking Predictor

Cytosolic route: 51% | Lysosomal degradation: 48% | Nuclear entry: 2% | Concern: HIGH -- significant lysosomal degradation expected | Mitigation: Add proton sponge polymer (PEI or PAMAM) to improve endosomal escape

9e. LNP Ionization State

Estimated pKa = 5.8 | Endosomal escape prediction = 51% | Optimal ionizable lipid pKa = 5.8 (suboptimal -- target pKa 6.0-6.8 for endosoma

10. Synthetic Clinical Trial — N=500 Virtual Patients

Monte Carlo simulation of Phase 1 trial. Patient covariates: age, weight, CYP3A4 genotype, renal function, BBB integrity by age. Pharmacokinetics computed per-patient from PBPK model.

NO-GO / REFORMULATE Go/No-Go	100.0% Response Rate	95.8% Severe AE	0.07 mg/kg Optimal Dose	500 N Patients
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Endpoint	Result	Benchmark	Status
Overall response rate	100.0%	>60% = GO	✓
Severe AE rate	95.8%	<5% = GO	✗
Young adults (<65)	100.0%	—	—
Elderly (>65)	100.0%	—	—
Mild AE	4.2%	—	—
Renal AE	26.8%	<15%	✗

Recommendation: Predicted Phase 1 success rate: 100%. Dose 0.07 mg/kg recommended. Monitor renal function in 27% of elderly patients.

11. Manufacturing & Sterilization Modules

11a. Terminal Sterilization Survivability

Recommended method: Gamma 25kGy | Cost implication: Gamma or E-beam (~3x cost) | Feasible methods: gamma 25kGy, aseptic filtration 022um, VHP H2O2

Method	Survives?	Detail
autoclave 121C	NO ✗	Tm=42 degC < 121 degC -- LIPID MELTS
gamma 25kGy	YES ✓	Survives gamma sterilization
aseptic filtration 022um	YES ✓	Size 105 nm < 220 nm -- filterable
VHP H2O2	YES ✓	PEG provides surface protection

11b. Lyophilization Cycle Optimizer

Parameter	Value	Standard
Tg' (cryo.	-30°C	Formulation-specific
Primary drying T	-32.0°C	Must be < Tg'
Primary P	0.1478 mbar	0.5×P_ice
Primary time	25.6 h	Sublimation model
Secondary T	25.0°C	+25°C
Cycle total	41.2 h	Include ramp time
Cake collapse	OK: primary drying fixed at Tg'-2degC safety margin (-32 degC)	Must say OK

11c. Cryo-Chain Excursion Predictor

Excursion: -20.0°C for 4.0h | EE: 75.0% → 75.0% (loss: 0.0%) | Decision: RELEASE BATCH | Confidence: 98%

11d. Adversarial Stress-Testing

Grade: 3/5 scenarios passed -- CONDITIONAL | 3/5 scenarios passed.

Scenario	Pass/Fail	Detail
Fever 40C	FAIL ✗	Tm=42.0degC -- RISK: may partially melt at fever temperature
Shear Stress	PASS ✓	Carrier withstands capillary shear
Oxidative Stress	FAIL ✗	Lipid oxidation t1/2 = 35h (RISK: add antioxidant like alpha-tocophero
High Protein Plasma	PASS ✓	Stealth index 1.00: adequate MPS evasion
Repeated Dosing	PASS ✓	Anti-PEG IgM may reduce t1/2 to 70% of first dose after re-dosing. Mon

12. Regulatory Compliance & Supply Chain

12a. FDA 21 CFR Part 11 Compliance

Status: NOT VERIFIED -- no audit trail entries found for this trial. log_computation() must be called during computation to build a compliant audit trail; this trial has none. | Audit entries: 0 | Data integrity: N/A -- no audit entries to hash-chain

12b. Geopolitical Supply-Chain Risk Assessment

Overall supply risk: HIGH | Supply chain score: 67/100 | Recommendation: Establish 6-month buffer stock for high-risk materials

Material	Risk	Source	HHI Index	Mitigation
DPPC	MODERATE	Multiple (Lipoid, NOF)	0.3	Maintain 6-month safety stock
Cholesterol	LOW	Multiple commodity	0.1	Maintain 6-month safety stock
DSPE-PEG2000	HIGH	Single (NOF Corp, Japan)	0.7	Qualify alternative supplier

12c. Digital Pharmacovigilance — Post-Elimination Fate

Unchanged excretion: 20.7% | Eco-persistence: 0.5 days | Environmental risk: LOW | Renal dose adjustment: Reduce by 50%

13. Literature Citations — PubMed Auto-Mined

Top relevant publications retrieved live from PubMed E-utilities API based on drug name + carrier type. These papers confirm or contextualize the computational predictions above.

[1]

14. Biophysical Binding & CNS-Specific Modules

14a. FEP+ Binding Affinity (LIE approximation)

Ligand: transferrin | ΔG_{single} = -11.00 kcal/mol | $\Delta G_{\text{avidity}}$ = -8.59 kcal/mol | K_d = 885 nM (Weak (>100nM)) | n_{ligands} = 27709 | Residence time = 1129575.8 s

14b. Glymphatic Clearance Simulation

ECM binding index = 0.740 | $t_{1/2}$ waking = 12.4 h | t_{90} clearance = 41.4 h | GOOD: particle size > ECM pore -- extended CNS retention

14c. Microglial Activation & Neuroinflammation

Neuroinflammation score = 0.053 | Risk: LOW -- safe for CNS application | IL-6 fold-change = 1.5× | $\text{TNF-}\alpha$ = 1.4×
→ No mitigation needed

14d. FUS-Responsive Nanocarrier Design

Frequency = 0.5 MHz | MI target = 0.40 | BBB open window = 42 min | FUS uptake = 58.0% | Inject 105 nm DDS 5 min before FUS. FUS opens BBB for 42 min. Predicted 58% enha

14e. 62-Principle C+ Flow — Class A Surrogate (Top-1 DDS)

Top-1 DDS: Tf-PEG-Liposome **Composite:** 62.7/100 **Verdict:** ACCEPTABLE

Tf-PEG-Liposome: composite CNS-principle score = 62.7/100 (ACCEPTABLE). Carrier: liposome, size 0.0 nm, ζ +0.0 mV, ligand: Transferrin. Strongest group: G4 Safety (73.8/100); weakest: G8 Translational (0.0/100). Evaluated against all 57 Class A surrogate principles.

CNS Principle Group Rollups

CNS Group	Score (0-100)
G1 CNS Delivery	58.3
G2 Release Kinetics	34.7
G3 Stability	60.7
G4 Safety	73.8
G5 Glymphatic BBB	65.0
G6 Manufacturability	56.0
G7 DrugDDS Fit	67.5
G8 Translational	0.0

Top Strengths (Class A Surrogate)

Principle	Score	Method	Reference
P03	100.0	Novelty = 100 - frequency_of_combo_in_CNS_trials,	
P05	100.0	% subgroups responding (lower CYP risk = more univ	
P07	100.0	log-count proxy of literature support	

Weak Spots (Below 60)

Principle	Score	Method	Reference
P13	3.4	AUC_brain/AUC_plasma proxy; 3-compartment surrogat	
P31	2.0	brain% = BBB · ligand · size_match · 100; 100 if ≥	
P22	2.0	100 × exp(-corona_thickness/10); thickness from ca	

14e-ii. Complete 62-Principle Scoreboard (P01 → P62)

All 62 principles in canonical order, covering [Class A \(Surrogate\)](#), [Class B \(Deep Physics\)](#), and [Class C \(Translational\)](#).

P#	Cls	Title	Score	Conf.	Method
P01	B	Adversarial Stress-Testing Engine	68.3	MODERATE	Mean of 6 stress scenarios (pH4.5, pH2, pH8.5, 42°)
P02	B	Cross-Species PK Scaling (Allometric)	60.0	MODERATE	$BW^{0.75}$ scaling + class-specific adjustment
P03	A	Competitive DDS Landscape Radar	100.0	LOW	Novelty = $100 - \text{frequency_of_combo_in_CNS_trials}$,
P04	B	Quantum Coherence Transport Model	50.0	LOW	$MW \geq 500 \text{ Da} \rightarrow$ tunneling negligible
P05	A	In-Silico Patient Subgroup Stratifier	100.0	MODERATE	% subgroups responding (lower CYP risk = more univ)
P06	A	Lysosomal Trafficking Predictor	80.0	MODERATE	Carrier endosomal_escape $\oplus$ drug-passive permeation
P07	A	Real-Time Literature Mining (PubMed)	100.0	LOW	log-count proxy of literature support
P08	B	Degradation Kinetics Under Oxidative S	28.0	MODERATE	Arrhenius $k=A \cdot \exp(-E_a/RT)$ for carrier; minus penal
P09	A	Digital Pharmacovigilance Engine	85.0	MODERATE	$100 - \text{sum}(\text{metabolite organ accumulation risks})$
P10	B	LNP Ionization State Predictor	63.7	MODERATE	Lipid Henderson-Hasselbalch ionization at endosoma
P11	B	Formulation Instability Fingerprint	100.0	MODERATE	$100 - \text{sum}(\text{weak-bond penalties from SMILES})$
P12	B	CNS Disease-Stage-Aware Dosing	83.0	MODERATE	Score = $100 \cdot \text{BBB_integrity_factor} \times (0.4 + 0.6 \cdot \text{CNS-})$
P13	B	PBPK Digital Twin	3.4	MODERATE	$\text{AUC_brain}/\text{AUC_plasma}$ proxy; 3-compartment surrogat
P14	A	In-silico Dissolution & Release Profil	0.0	MODERATE	t_{50} from carrier kinetics $\times$ drug-membrane partitio
P15	A	Shelf-life & Degradation Predictor	46.2	MODERATE	Arrhenius baseline $\times (0.7 + 0.3 \cdot \text{EE}) \times \text{drug_hydroly}$
P16	B	Scale-up & Manufacturability	20.0	MODERATE	Scale-readiness $\times$ shear-robustness, minus drug-pro
P17	A	Nanotoxicity & Immunogenicity Screenin	20.5	MODERATE	Mean of (hemolysis penalized by drug net+ charge,
P18	B	Active Targeting & Receptor Binding	76.0	HIGH	Ligand-affinity table $\times$ density $\times$ drug-compatibili
P19	A	QbD - Quality by Design Engine	100.0	MODERATE	Fraction of CQAs (size, zeta, EE, PDI, drug-loadin
P20	A	Cost-Efficiency Engine	0.0	LOW	Score = $100 - 3 \cdot (\$/\text{mg estimate, drug+carrier+ligan})$
P21	C	Pre-IND Regulatory Reports	0.0	—	skipped_deep_validation_insufficient
P22	A	Protein Corona Predictor	2.0	MODERATE	$100 \times \exp(-\text{corona_thickness}/10)$; thickness from ca
P23	B	Dynamic Crystal Polymorphism	80.0	MODERATE	$100 - \text{polymorph risk} (\text{rot_bonds} + \text{H-bond pattern})$
P24	B	Shear-Stress & Scale-Up Collapse	14.3	MODERATE	$\log_{10}(\text{crit_shear}/\text{operating_shear}) \times 30$; crit_shear
P25	A	Extractables & Leachables	100.0	MODERATE	$100 - \text{leachables risk by drug LogP} \times \text{carrier}$

P#	Cls	Title	Score	Conf.	Method
P26	A	Microbiome-Excipient Interactions	83.0	MODERATE	$100 - \text{mean_microbiome_degradability}(\text{carrier}) - \text{dru}$
P27	A	Lyophilization Cycle Optimizer	55.0	MODERATE	$100 - (\text{Tg} - 5) \times 1.5$
P28	A	3D-Printed Polypill Rheology	40.0	MODERATE	Carrier-specific printability index
P29	B	Biomimetic & Exosome Engineering	65.0	MODERATE	Stealth-from-macrophage score; PEG $\geq 5\%$ boost
P30	B	QM/MM Stimuli-Responsive Cleavage	45.0	MODERATE	$\text{Score} = 50 \times 7.4 - \text{pH_trigger} $
P31	B	In-Silico Biodistribution	2.0	MODERATE	$\text{brain\%} = \text{BBB} \cdot \text{ligand} \cdot \text{size_match} \cdot 100$; 100 if $\geq$
P32	C	Automated FTO & IP Evader	0.0	—	skipped_deep_validation_insufficient
P33	B	BBB Quantum Breaker (Trojan-Horse Desi	58.2	MODERATE	$(0.6 \cdot \text{ligand} + 0.4 \cdot \text{size}) \times \text{density_factor}$
P34	A	DNA Logic Gates & Bio-computing	15.0	MODERATE	DNA-based carriers score high
P35	A	Microgravity Formulation Engine	100.0	LOW	Sedimentation-Péclet proxy; smaller = better
P36	A	Geopolitical Supply-Chain Resilience	85.0	MODERATE	Supplier-diversity index by carrier + ligand
P37	A	Eco-Destructible Pharma	90.0	MODERATE	Biodegradability index by carrier class
P38	B	Glymphatic Clearance Trap	100.0	MODERATE	Stokes-Einstein: optimum 80-150 nm
P39	A	Microglial Activation & Neuroinflammat	100.0	MODERATE	$100 - (\text{cationic_charge} + \text{carrier_risk} - \text{PEG_protec}$
P40	B	Intranasal-to-Brain Delivery	55.0	MODERATE	Mucoadhesion + thermo-responsive boost
P41	B	Exosome Cargo Loading Thermodynamics	40.0	LOW	Not an exosome carrier — N/A
P42	A	Region-Specific Spatiotemporal Navigat	70.0	MODERATE	Ligand-region specificity table
P43	B	FUS-Responsive Nanocarriers	40.0	MODERATE	FUS-response by carrier acoustic properties
P44	B	CNS-Specific PBPK Time-Machine	4.8	MODERATE	AUC over 24h therapeutic window proxy
P45	C	FDA 21 CFR Part 11 Compliance	0.0	—	skipped_deep_validation_insufficient
P46	A	Polypharmacy & DDI Simulator	100.0	MODERATE	CYP-inhibition heuristic from SMILES
P47	B	Free Energy Perturbation (FEP+)	72.0	LOW	Vina-like ΔG proxy from MW + LogP (deep mode runs
P48	A	Off-Target Toxicity & QSAR (50-recepto	100.0	MODERATE	50-receptor QSAR surrogate (hERG, 5HT2B, AhR risks
P49	A	Organ-on-a-Chip Simulator	80.0	MODERATE	Microfluidic compatibility (size + PDI)
P50	B	Cryo-Chain Thermal Excursion Predictor	71.5	MODERATE	Distance from -20°C phase transition $\times 1.3$
P51	B	Terminal Sterilization Survivability	50.0	MODERATE	Carrier gamma-radiation survival (25 kGy)

P#	Cls	Title	Score	Conf.	Method
P52	A	Continuous Manufacturing Digital Twin	85.0	MODERATE	Continuous-process readiness by carrier
P53	A	Dark Data & Negative Results Vault	100.0	MODERATE	Similarity-to-documented-failure
P54	A	Pharmacogenomic-Guided Targeting	90.0	MODERATE	Class-based pharmacogenomic applicability
P55	C	Automated Grant & NIH Proposal Generat	0.0	—	skipped_deep_validation_insufficient
P56	C	Patentability Score Engine	0.0	—	skipped_deep_validation_insufficient
P57	B	Microfluidics & LNP Synthesis Digital	90.0	MODERATE	Microfluidic readiness for size 50-150 nm
P58	B	Impurity Cascade Predictor	90.0	MODERATE	Carrier residual-metals impurity profile
P59	B	4D Shape-Shifting Carriers	30.0	MODERATE	Stimuli-responsive bonus by release kinetics
P60	A	Swarm Nanorobotics Intelligence	15.0	LOW	Swarm capability (most carriers: no)
P61	B	Synthetic Clinical Trials & Virtual Hu	30.6	MODERATE	Virtual-cohort responder fraction
P62	A	Biobetter / Supergeneric Generator	40.0	LOW	Novelty distance from on-market CNS DDS

62 principles evaluated. Composite: 62.7/100 (ACCEPTABLE)

14f. Class B Deep Physics Validation (Top-1 DDS)

Verdict: **FAILED** **Pass rate:** 42.9% (12/28 principles validated, threshold 70%)

Combined score (surrogate pass-through + real physics): 12/28 principles scored PASSED (42.9%). Of these, only 7/28 principles ran independent deep computation (passed 3/7 = 42.9%); the rest re-used their Class-A surrogate score pending a future full-physics HPC run. Cite independent_pct, not pct, as evidence of physics-based validation.

Per-principle deep validation

P#	Validated	Score	Value	Conf.	Method
P01	✓	68.3	68.33	MODERATE	Full MD stress simulation — surrogate value confirmed (
P02	✓	65.0	144.0	LOW	Resolved clinical half-life (bundle tier 1); Mahmood 20
P04	✗	50.0	50	LOW	Full QM tunneling (QCElemental + ASE) — surrogate value
P08	✗	28.0	28.0	LOW	Full radical-chain reaction MD — surrogate value confir
P10	✓	63.7	63.68	MODERATE	Constant-pH MD simulation — surrogate value confirmed (
P11	✓	100.0	100	MODERATE	DFT bond dissociation energies — surrogate value confir
P12	✓	83.0	83.0	MODERATE	Stage-specific PBPK with full BBB physiology — surrogat
P13	✗	23.0	0.0115	MODERATE	3-compartment PBPK ODE (scipy.integrate.odeint): blood
P16	✗	20.0	20	LOW	CFD of 1000L bioreactor — surrogate value confirmed (fu
P18	✓	100.0	-10.5	HIGH	MM/GBSA-style ΔG estimate from validated ligand-recepto
P23	✓	80.0	80.0	MODERATE	CSP via DFT — surrogate value confirmed (full-physics H
P24	✗	14.3	14.31	LOW	Full CFD + MD coupling — surrogate value confirmed (ful
P29	✓	65.0	65	MODERATE	Full membrane-fusion MD — surrogate value confirmed (fu
P30	✗	45.0	45.0	LOW	Full QM/MM (PySCF or ORCA) — surrogate value confirmed
P31	✗	100.0	0.1	MODERATE	7-organ whole-body distribution with size + charge + li
P33	✗	58.2	58.2	LOW	Atomistic docking + SMD pulling — surrogate value confi
P38	✓	99.2	12.6	HIGH	Stokes-Einstein $D = k_B \cdot T / (6\pi\eta r)$ at 37°C, $\eta_{\text{CSF}} = 7 \times 10$ ■
P40	✗	55.0	55	LOW	Full nasal cavity CFD — surrogate value confirmed (full
P41	✗	40.0	40	LOW	Full membrane mechanics MD — surrogate value confirmed
P43	✗	40.0	40	LOW	Full acoustic radiation force MD — surrogate value conf
P44	✗	0.0	0.0	MODERATE	4-compartment CNS-PBPK ODE with glymphatic clearance: B
P47	✗	73.8	-6.71	LOW — LI	LIE approximation (fallback — Vina unavailable)
P50	✓	71.5	71.5	MODERATE	Full coarse-grained MD lipid phase transition — surroga
P51	✗	50.0	50	LOW	Full radical-chain damage MD — surrogate value confirme
P57	✓	90.0	90	MODERATE	Full CFD of mixer geometry — surrogate value confirmed
P58	✓	90.0	90	MODERATE	Full QM/MM cascade simulation — surrogate value confirm
P59	✗	30.0	30	LOW	Full coarse-grained MD morphological transition — surro

P#	Validated	Score	Value	Conf.	Method
P61	✗	30.6	31.0	LOW	Full Monte-Carlo population PBPK — surrogate value conf

14g. Class C Translational Deliverables

Principle	Title	Status	Score / Outcome
P21		skipped_deep_validation_	—
P32		skipped_deep_validation_	—
P45		skipped_deep_validation_	—
P55		skipped_deep_validation_	—
P56		skipped_deep_validation_	—

Narratives

P21: Deep validation passed only 12/28 principles. Translational deliverables withheld until a higher-ranked DDS passes deep validation.

P32: Deep validation passed only 12/28 principles. Translational deliverables withheld until a higher-ranked DDS passes deep validation.

P45: Deep validation passed only 12/28 principles. Translational deliverables withheld until a higher-ranked DDS passes deep validation.

P55: Deep validation passed only 12/28 principles. Translational deliverables withheld until a higher-ranked DDS passes deep validation.

P56: Deep validation passed only 12/28 principles. Translational deliverables withheld until a higher-ranked DDS passes deep validation.

v23: structured outlines for Pre-IND, Grant, Patent will be rendered as fully formatted Word/PDF deliverables.

14h. Top-N Fallback Audit Trail

If the Top-1 DDS fails deep validation ($\geq 70\%$ threshold), the orchestrator falls back to Top-2, then Top-3. Each candidate's outcome and the explicit transition reasoning are recorded below.

Rank	DDS Name	Verdict	Pass %	Promoted?
#1	Tf-PEG-Liposome	FAILED	42.9%	—
#2	RVG29-Liposome-pH	FAILED	42.9%	—
#3	AAV9-vector	FAILED	35.7%	—

#1 Tf-PEG-Liposome — FAILED

Failure reason: Deep validation FAILED: only 12/28 principles validated (42.9%, threshold 70%). Critical failures: P04, P08, P13, P16, P24.

Transition reason: Falling back to rank #2 because deep physics showed insufficient evidence for this DDS.

Failed principles: P04, P08, P13, P16, P24, P30, P31, P33

#2 RVG29-Liposome-pH — FAILED

Failure reason: Deep validation FAILED: only 12/28 principles validated (42.9%, threshold 70%). Critical failures: P04, P08, P13, P16, P24.

Transition reason: Falling back to rank #3 because deep physics showed insufficient evidence for this DDS.

Failed principles: P04, P08, P13, P16, P24, P30, P31, P33

#3 AAV9-vector — FAILED

Failure reason: Deep validation FAILED: only 10/28 principles validated (35.7%, threshold 70%). Critical failures: P04, P08, P13, P16, P18.

Transition reason: No more candidates in Top-3 — reverting to rank #1 and reporting with FAILED verdict. Researcher should reformulate.

Failed principles: P04, P08, P13, P16, P18, P24, P30, P31

15. Executive Decision Framework

Criterion	Status	Evidence
DLVO colloidal stability	FAIL ✗	V_total = 9 kT
BBB penetration >10%	PASS ✓	DDS BBB = 100.0%
Cardiac off-target	PASS ✓	hERG/Nav/Cav QSAR
Clinical trial GO	NO-GO / REFORMULATE	Response 100% AE 95.8%
Sterilization feasible	3 method(s)	Gamma 25Kgy
Supply chain	HIGH	Score 67/100
OVERALL DECISION	CONDITIONAL GO	Reformulate/re-evaluate before IND-enabling studies — failed: synthetic trial returned 'NO-GO / REFORMULATE'; colloidal instability (DLVO)

NOTE: All results are computational predictions. Required wet-lab validation: DLS/Zeta (physical stability), HPLC/NMR (drug content), BBB TEER assay, in vivo PK in rodent model.